

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Academy of Neurology Annual Meeting (AAN) 2026

Meeting period: April 2026 | Author: Priya Menon

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Priya Raghavan, MD (North Star Neurology)

I met the physician at the Thursday registration desk, and the discussion concerned telehealth logistics only. Two operational points were raised: (1) video f/u works well for stable pts, although seizure-diary review is awkward without a shared screen, and (2) out-of-state pts have raised licensure questions that the group is still working through. I confirmed that no action from my side was required and closed the conversation there.

Conversation — Robert Enloe, MD (High Desert Neurology)

A short conversation followed the Thursday afternoon session, again on administrative burden rather than anything clinical. The substance was as follows: prior-authorization paperwork now consumes a considerable share of one staff member's week, appeals are frequently resubmitted for formatting reasons alone, and no product or payer was named at any point. I offered no materials, as the topic sits outside my remit.

Conversation — Grace Lindqvist, MD (Riverstone Medical Group)

I was approached during the Friday poster session; the conversation was unsolicited, and I did not review the poster itself beyond the title on display. Standing at the poster, asked why the adolescent drug-resistant population is still excluded, describing families sent away empty-handed until 18. I replied that I am able to discuss only the approved adult indication for XCOPRI, that I could not comment on any use below 18, and that the question would be routed to Medical Information. Closed by requesting whatever adolescent safety or pharmacokinetic data exists, published or pending.

Conversation — Jaromir Cavanaugh, MD (Granite Bay Epilepsy Center)

The conversation happened while waiting for a session room to clear. Benefit verification is the step that most often delays a start here. Explained that drug-drug interactions on top of already long medication lists in patients over 65. The composition of the surgical workup list has shifted over the last year or two.

Conversation — Zubin Abernathy, MD (Cedar Ridge Regional Epilepsy Program)

A hallway conversation during the afternoon break. Raised, without any prompting, the wait-until-18 pattern, called an uncomfortable way to practice. Referenced a case series someone forwarded in

passing. Asked me to send confirmation of whether any adolescent safety data is available yet.

Conversation — Kiara Vandergriff, MD (Beacon Hill Neurology Associates)

Introduced by a colleague from the same region and talked for a few minutes. Monitoring intervals during titration are set by clinic routine here rather than by protocol. Benefit checks takes longer than the clinical decision, which came up as a general frustration. The reluctance here is how the medication list in patients over 65 makes the whole decision feel heavier.

Follow-ups have been logged in the interaction system.